

Medication Adherence

CCG: C-1031 (CCG)

MCG Health
Chronic Care
28th Edition

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Assessment

Q1. Patient: Tell me what you know about each of your medicines. (Go through list for each medication and check all that apply.)^[A]

- What is the name of the medicine?
- What health problem are you taking the medicine for?
- How much of the medicine (number of pills) do you take each time?
- When are you supposed to take the medicine?
- What does this medicine do?
- What are the possible side effects of this medicine to watch for?
- Where do you keep this medicine?
- When is the next refill of this medicine due?
- Not all appropriate information known ^[P1]
- Unknown ^[P1]

Q2. Patient: Did you take all your medicine yesterday?

- Yes
- No ^[P1]
- Unknown ^[P1]

Q3. Patient: In the last 2 weeks, have you forgotten to take your medicine?

- Yes ^[P1]
- No
- Unknown ^[P1]

Q4. Patient: Are you careless about taking your medicine sometimes?

- Yes ^[P1]
- Never/rarely
- Unknown ^[P1]

Q5. Patient: Do you take your medicine at the same time every day?

- Yes
- Sometimes ^[P1]
- No ^[P1]
- Unknown ^[P1]

Q6. Patient: Are you taking injectable medicines?

- Yes ^[Q7]
- No
- Unknown ^[P2]

Q7. Patient: Do you give yourself injectable medicine?

- Yes ^[Q8]
- No, someone else does. Name and phone number of person giving medicine: _____
^[Q8]
- No ^[Q8] ^[P2]
- Unknown ^[P2]

Q8. Patient: Do you have any problems giving yourself an injection? (Check all that apply.)

- Caring for the injection site [P2]
- Choosing and preparing the injection site [P2]
- Disposing of syringes and needles [P2]
- Getting the medicine ready for injection [P2]
- Giving myself injections [P2]
- Storing the medicine properly [P2]
- Unknown [P2]
- I have no problem performing the procedures.

Q9. Patient: What keeps you from taking your medicine as prescribed? (Check all that apply.)

- Do not want to take the medicine [P1]
- Feel better, so do not need it anymore [P1]
- Feel worse after taking it [Q10] [P1]
- Not sure it helps [P1]
- Not sure it is safe to take [P1]
- Not sure what medicine to take and when [P1]
- Problems because medicine is not part of health plan or pharmacy formulary [P1]
- Problems getting refills on time [P1]
- Problems opening pill bottles or handling pills [P1]
- Problems paying for medicine [P1]
- Problems reading or understanding medicine bottles (due to vision, language, or literacy issues) [P1]
- Problems with swallowing pills [P1]
- Side effects [Q10] [P1]
- Unable to get to pharmacy [P1]
- Other, specify: _____ [P1]
- Unknown [P1]
- No issues keep me from taking my medicine.

Q10. Patient: Does the doctor who prescribed the medicine know about the side effects you are having?

- Yes
- No [P1]
- Unknown [P1]

Q11. Patient: Have you told the doctor you are not taking the medicine?

- Yes
- No [P1]
- Unknown [P1]

Q12. Patient: Has your mood changed over the past 2 weeks?(1)(2)  PHQ-2 Calculator

- Yes [Q13]
- No
- Unknown

Q13. Patient: What changes in your mood have you had? (Check all that apply.)

- Change in usual activity (eg, working excessively, no longer talking to friends, or going on shopping sprees) Depression Screening, Adult  CCG
- Down, hopeless, or depressed Depression Screening, Adult  CCG
- Forgetting a lot or getting angry or frustrated Depression Screening, Adult  CCG
- Little interest or pleasure in doing things Depression Screening, Adult  CCG
- Quick mood changes Depression Screening, Adult  CCG
- Other, specify: _____ Depression Screening, Adult  CCG
- Not applicable

Q14. Patient: Are there any personal considerations affecting your ability to take medicine? (Check all that apply.)

- Ability to afford [P1] Financial Status and Benefits  CCG
- Cultural or religious [P1] Cultural Practices  CCG
- Forgetfulness [P1] Cognitive Impairment  CCG

- Health literacy [P1] Health Literacy  CCG
- Need for help from someone else when taking medicine [P1] Caregiver Resources Evaluation  CCG
- Transportation to and from pharmacy [P1] Transportation  CCG
- Traveling with medicine [P1] Travel  CCG
- Other, specify: _____ [P1]
- None
- Unknown [P1]

Plan of Care

Problems

P1. Medication adherence improvement needed

- **Goal:** Adherence to medication regimen improved[B]
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling:
 - Understanding preferences for medication use (eg, wanting to take fewer pills) to discuss with provider
 - Understanding behavioral health aspects and how to get help
 - **ASSIST:** Identifying and creating a plan to manage adherence issues (current or potential) that interfere with following prescribed medication regimen(3)(4):
 - Resolving common medication adherence barriers (eg, difficulty remembering how or when to take, difficulty understanding labels, side effects, costs)(4)
 - Identifying ways to improve following medication routine
 - Overcoming communication barriers (eg, vision, language), as needed
 - Simplifying medication regimen (eg, administration once or twice daily rather than more often)(4)
 - Identifying medication discrepancies and resolving with provider
 - Determining when refills are due and developing a plan for obtaining refills
 - Working with pharmacy for filling prescriptions (eg, changing pills to liquids, identifying financial support, delivery)
 - Follow-up appointment scheduling to evaluate treatment outcomes
 - **ASSIST:** Communication with treating provider when another provider (eg, emergency or specialty provider) prescribes new medication
 - **EDUCATE:** Medication routine improvement techniques(4):
 - Organize medications (written charts, calendar trays with day and time of each dose, pill and bottle counts).
 - Reminder system to help with when and how to take medication and when refills are due(5)
 - Fill all prescriptions at the same pharmacy.
 - Add medication regimen to daily habits (eg, with breakfast, before bedtime routine).
 - Review pharmacy medication information (precautions, taking with or without meals, side effects to contact provider about).(5)
 - When and how to get help for problems with medication
 - Use of generic instead of brand-name medication(3)
 - Psychosocial and environmental influences(6)
 - **EDUCATE:** Available financial aid resources (pharmacy programs, discounts for quarterly refills)
 - **EDUCATE:** Pharmacy assistance programs, such as:
 - Centers for Medicare and Medicaid Services (CMS): www.medicare.gov/drug-coverage-part-d
 - Medicine Assistance Tool: <https://medicineassistancetool.org>
 - NeedyMeds: www.needymeds.org
 - Individual company websites
 - **EDUCATE: Reason for taking medication(7):**
 - Expected effects if the medication is working, and expected time for effects to occur
 - How long medication will likely need to be taken
 - What happens if medication is not taken as prescribed
 - **EDUCATE:** Nonadherence may cause(4):
 - Continued or worsening health problems
 - Increased costs (need to take more medication, risk of hospitalization or long-term care)(5)
 - Increased burden on caregivers
 - Decreased quality of life
 - **EDUCATE:** Information on all medications(7)(8)(9)(10):

- Medication purpose, dose, effects/side effects, and considerations (eg, managing missed doses, interactions with food/other drugs)
- Name of provider who ordered medication
- Administration techniques
- Side effects, related management, and when to contact provider
- **Get emergency help:** Adverse or allergic reactions (eg, wheezing, chest tightness, fever, itching, bad cough, blue skin color, hemorrhage, seizures, swelling of face, lips, tongue, or throat)
- Common side effects include gastrointestinal changes, rash, dizziness, and headache.
- Do not stop long-term medications abruptly due to risk of withdrawal symptoms.
- Medication safety (eg, proper storage and disposal)(11)
- Monitoring (eg, vital signs, laboratory checks) needed while taking medication
- Talk to provider before taking any over-the-counter medications, vitamins, minerals, or herbal or dietary supplements.
- Carry a list of medications at all times, particularly when visiting healthcare providers.
- **COORDINATE:** Clinical monitoring appropriate to medication use (eg, laboratory tests)
- **COORDINATE:** Communication between care team members and medication list reconciliation.(10)(12) See Medication Reconciliation Tool  SR for more information.
- **COORDINATE:** Communication between patient and pharmacy (for obtaining prescriptions, setting up home medication delivery, review for drug-drug interactions)(13)
- **COORDINATE:** Obtaining medication organizer or reminder tools (eg, pillbox)
- **COORDINATE:** Behavioral health services referral, if indicated
- **COORDINATE:** Community services referral, as needed
- **COORDINATE:** Home care referral, if indicated
- **COORDINATE:** Social worker referral, as needed(14)
- **COORDINATE:** Medication pickup by family/friends
- **COORDINATE:** Communication between care team members and reconciled medication list(12)
- **COORDINATE:** Follow-up to evaluate adherence or problems with medication, provide additional education and support, and identify need for further assistance(15)(16)
- **SEND:** Managing your medicines (Illustrated)  (Spanish)

P2. Injectable medication adherence improvement needed

- **Goal:** Injectable medication regimen adherence improved
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling with provider for discussion on problems with injectable medication administration or use
 - **ASSIST:** Creating a plan to manage issues (current or potential) that interfere with injectable medication regimen:
 - Finding support to help with injectable medication management
 - Overcoming communication barriers (eg, vision, language), as needed
 - **EDUCATE:** Techniques to safely give injectable medication(17):
 - Supplies needed (eg, syringes, needles) and safe use
 - Administration techniques (caregiver should administer medication if patient unable to)
 - Site rotation
 - **EDUCATE:** Information on injectable medications (eg, purpose, dose, timing)(9)
 - **COORDINATE:** Communication with pharmacy (eg, home delivery, change in medication form, help with drug-drug or food-drug problems)
 - **COORDINATE:** Home care referral, if indicated
 - **COORDINATE:** Social worker referral, as needed(14)
 - **COORDINATE:** Follow-up contact to evaluate injectable medication use and need for further assistance(15)(16)
 - **SEND:** Giving yourself an injection (Illustrated)  (Spanish)

Evidence Summary

Evidence: A cross-sectional analysis of 4427 older adults found that lack of prescription drug coverage, ADL limitations, and depression were all significant factors affecting cost-related medication nonadherence.(18)

Evidence: A retrospective analysis of over 400,000 adults with chronic illness found that medication adherence was associated with lower odds of hospitalization and emergency department use compared to nonadherent patients.(19)

Evidence: In a randomized controlled trial, 5216 patients with a first-time statin prescription who did not pick up the medication within 1 week received a telephone reminder and personalized letter emphasizing the importance of therapy to prevent heart attack and stroke; 17% more patients in the intervention group filled the initial prescription within 2 weeks of the reminder call compared to patients in the control group.(20)

Evidence: A systematic review of 29 studies found that social adversities (eg, food insecurity, housing instability) are associated with lower medication adherence.(6)

Outcomes: Several studies support the role of the pharmacist in medication reconciliation to identify and resolve pharmacy-related problems and improve outcomes (eg, prevention of medication-related adverse events, discontinuation of inappropriate medication).(21)

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Footnotes

[A] Having the patient's reconciled medication list ready before this interview can help identify knowledge gaps. See Medication Reconciliation Tool [SR](#) for more information. [A in Context Link 1]

[B] There are different classifications of nonadherence, including primary (eg, the patient does not fill the prescription); nonpersistence (eg, the medications were dispensed but not refilled); preventable (eg, the patient forgets to take the medication); and nonpreventable (eg, adverse reactions when taking the drug).(22)(23) The acceptable medication adherence level may vary depending on the disease phenotype (eg, medication adherence level during times of exacerbation vs annual adherence level).(24) [B in Context Link 1]

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